

1 **HOUSE OF REPRESENTATIVES - FLOOR VERSION**

2 STATE OF OKLAHOMA

3 2nd Session of the 60th Legislature (2026)

4 ENGROSSED SENATE
5 BILL NO. 1642

By: Frix of the Senate

and

Marti of the House

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11 An Act relating to controlled dangerous substances;
12 amending 63 O.S. 2021, Section 2-309I, as amended by
13 Section 1, Chapter 257, O.S.L. 2022 (63 O.S. Supp.
14 2025, Section 2-309I), which relates to prescription
15 limits and rules for opioid drugs; authorizing
16 divided quantities for certain acute pain
17 prescriptions; updating statutory language; modifying
18 statutory references; and providing an effective
19 date.

20 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

21 SECTION 1. AMENDATORY 63 O.S. 2021, Section 2-309I, as
22 amended by Section 1, Chapter 257, O.S.L. 2022 (63 O.S. Supp. 2025,
23 Section 2-309I), is amended to read as follows:
24

1 Section 2-309I. A. ~~A practitioner shall not issue an initial~~
2 ~~prescription for an opioid drug in a quantity exceeding a seven-day~~
3 ~~supply for treatment of acute pain. Any opioid prescription for~~
4 ~~acute pain shall be for the lowest effective dose of an immediate-~~
5 ~~release drug.~~

6 ~~B.~~ Prior to issuing an initial prescription for an opioid drug
7 in a course of treatment for acute or chronic pain, a practitioner
8 shall:

9 1. Take and document the results of a thorough medical history,
10 including the experience of the patient with nonopioid medication
11 and nonpharmacological pain-management approaches and substance
12 abuse history;

13 2. Conduct, as appropriate, and document the results of a
14 physical examination;

15 3. Develop a treatment plan with particular attention focused
16 on determining the cause of pain of the patient;

17 4. Access relevant prescription monitoring information from the
18 central repository pursuant to Section 2-309D of this title;

19 5. ~~Limit the supply of any opioid drug prescribed for acute~~
20 ~~pain to a duration of no more than seven (7) days as determined by~~
21 ~~the directed dosage and frequency of dosage; provided, however, upon~~
22 ~~issuing an initial prescription for acute pain pursuant to this~~
23 ~~section, the practitioner may issue one (1) subsequent prescription~~
24 ~~for an opioid drug in a quantity not to exceed seven (7) days if:~~

- 1 a. ~~the subsequent prescription is due to a major surgical~~
2 ~~procedure or "confined to home" status as defined in~~
3 ~~42 U.S.C., Section 1395n(a),~~
4 b. ~~the practitioner provides the subsequent prescription~~
5 ~~on the same day as the initial prescription,~~
6 c. ~~the practitioner provides written instructions on the~~
7 ~~subsequent prescription indicating the earliest date~~
8 ~~on which the prescription may be filled, otherwise~~
9 ~~known as a "do not fill until" date, and~~
10 d. ~~the subsequent prescription is dispensed no more than~~
11 ~~five (5) days after the "do not fill until" date~~
12 ~~indicated on the prescription;~~

13 ~~6.~~ In the case of a patient under the age of eighteen (18)
14 years, enter into a patient-provider agreement with a parent or
15 guardian of the patient; and

16 ~~7.~~ 6. In the case of a patient who is a pregnant woman, enter
17 into a patient-provider agreement with the patient.

18 B. 1. A practitioner shall not issue an initial prescription
19 for an opioid drug for treatment of acute pain in a quantity
20 exceeding a seven-day supply, as determined by the directed dosage
21 and frequency of dosage.

22 2. Any initial or subsequent opioid prescription for acute pain
23 shall be for the lowest effective dose of an immediate-release drug.
24

1 3. The practitioner may issue the initial seven-day
2 prescription in divided quantities, which shall only count as a
3 single prescription for purposes of the requirements of this
4 section.

5 C. ~~No~~ Except as provided in subsection D of this section, no
6 less than seven (7) days after issuing the initial acute pain
7 prescription pursuant to subsection ~~A~~ B of this section, the
8 practitioner, after consultation with the patient, may issue a
9 subsequent acute pain prescription for the opioid drug to the
10 patient in a quantity not to exceed seven (7) days, provided that:

11 1. The subsequent prescription would not be deemed an initial
12 prescription under this section;

13 2. The practitioner determines the prescription is necessary
14 and appropriate to the treatment needs of the patient and documents
15 the rationale for the issuance of the subsequent prescription; and

16 3. The practitioner determines that issuance of the subsequent
17 prescription does not present an undue risk of abuse, addiction or
18 diversion and documents that determination.

19 D. 1. The practitioner may issue the subsequent seven-day
20 acute pain prescription under subsection C of this section in
21 divided quantities, which shall only count as a single prescription
22 for purposes of the requirements of this section.

23 2. Notwithstanding the timing and quantity restrictions
24 specified in subsection C of this section, upon issuing an initial

1 prescription of an opioid drug for acute pain under subsection B of
2 this section, the practitioner may simultaneously issue one
3 subsequent prescription for an opioid drug in a quantity not to
4 exceed seven (7) days if:

- 5 a. the subsequent prescription is due to a major surgical
6 procedure or confined to home status as described in
7 42 U.S.C., Section 1395n(a),
- 8 b. the practitioner provides the subsequent prescription
9 on the same day as the initial prescription,
- 10 c. the practitioner provides written instructions on the
11 subsequent prescription indicating the earliest date
12 on which the prescription may be filled, otherwise
13 known as a "do not fill until" date, and
- 14 d. the subsequent prescription is dispensed no more than
15 five (5) days after the "do not fill until" date
16 indicated on the prescription.

17 E. Prior to issuing the initial prescription of an opioid drug
18 in a course of treatment for acute or chronic pain and again prior
19 to issuing the third prescription of the course of treatment, a
20 practitioner shall discuss with the patient or the parent or
21 guardian of the patient if the patient is under eighteen (18) years
22 of age and is not an emancipated minor, the risks associated with
23 the drugs being prescribed, including, but not limited to:

1 1. The risks of addiction and overdose associated with opioid
2 drugs and the dangers of taking opioid drugs with alcohol,
3 benzodiazepines and other central nervous system depressants;
4 2. The reasons why the prescription is necessary;
5 3. Alternative treatments that may be available; and
6 4. Risks associated with the use of the drugs being prescribed,
7 specifically that opioids are highly addictive, even when taken as
8 prescribed, that there is a risk of developing a physical or
9 psychological dependence on the controlled dangerous substance, and
10 that the risks of taking more opioids than prescribed or mixing
11 sedatives, benzodiazepines or alcohol with opioids can result in
12 fatal respiratory depression.

13 The practitioner shall include a note in the medical record of
14 the patient that the patient or the parent or guardian of the
15 patient, as applicable, has discussed with the practitioner the
16 risks of developing a physical or psychological dependence on the
17 controlled dangerous substance and alternative treatments that may
18 be available. The applicable state licensing board of the
19 practitioner shall develop and make available to practitioners
20 guidelines for the discussion required pursuant to this subsection.

21 ~~E.~~ F. At the time of the issuance of the third prescription for
22 an opioid drug, the practitioner shall enter into a patient-provider
23 agreement with the patient.
24

1 ~~F.~~ G. When an opioid drug is continuously prescribed for three
2 (3) months or more for chronic pain, the practitioner shall:

3 1. Review, at a minimum of every three (3) months, the course
4 of treatment, any new information about the etiology of the pain,
5 and the progress of the patient toward treatment objectives and
6 document the results of that review;

7 2. In the first year of the patient-provider agreement, assess
8 the patient prior to every renewal to determine whether the patient
9 is experiencing problems associated with an opioid use disorder as
10 defined by the American Psychiatric Association and document the
11 results of that assessment. Following one (1) year of compliance
12 with the patient-provider agreement, the practitioner shall assess
13 the patient at a minimum of every six (6) months;

14 3. Periodically make reasonable efforts, unless clinically
15 contraindicated, to either stop the use of the controlled substance,
16 decrease the dosage, or try other drugs or treatment modalities in
17 an effort to reduce the potential for abuse or the development of an
18 opioid use disorder as defined by the American Psychiatric
19 Association and document with specificity the efforts undertaken;

20 4. Review the central repository information in accordance with
21 Section 2-309D of this title; and

22 5. Monitor compliance with the patient-provider agreement and
23 any recommendations that the patient seek a referral.
24

1 ~~G.~~ H. 1. Any prescription for acute pain pursuant to this
2 section shall have the words "acute pain" notated on the face of the
3 prescription by the practitioner.

4 2. Any prescription for chronic pain pursuant to this section
5 shall have the words "chronic pain" notated on the face of the
6 prescription by the practitioner.

7 ~~H.~~ I. This section shall not apply to a prescription for a
8 patient:

9 1. Who has sickle cell disease;

10 2. Who is in treatment for cancer or receiving aftercare cancer
11 treatment;

12 3. Who is receiving hospice care from a licensed hospice;

13 4. Who is receiving palliative care in conjunction with a
14 serious illness;

15 5. Who is a resident of a long-term care facility; or

16 6. For any medications that are being prescribed for use in the
17 treatment of substance abuse or opioid dependence.

18 ~~I.~~ J. Every policy, contract, or plan delivered, issued,
19 executed, or renewed in this state, or approved for issuance or
20 renewal in this state by the Insurance Commissioner, and every
21 contract purchased by the Employees Group Insurance Division of the
22 Office of Management and Enterprise Services, on or after November
23 1, 2018, that provides coverage for prescription drugs subject to a
24 copayment, coinsurance or deductible shall charge a copayment,

1 coinsurance, or deductible for an initial prescription of an opioid
2 drug prescribed pursuant to this section that is either:

3 1. Proportional between the cost sharing for a thirty-day
4 supply and the amount of drugs the patient was prescribed; or

5 2. Equivalent to the cost sharing for a full thirty-day supply
6 of the drug, provided that no additional cost sharing may be charged
7 for any additional prescriptions for the remainder of the thirty-day
8 supply.

9 ~~J.~~ K. Any practitioner authorized to prescribe an opioid drug
10 shall adopt and maintain a written policy or policies that include
11 execution of a written agreement to engage in an informed consent
12 process between the prescribing practitioner and qualifying opioid
13 therapy patient. For the purposes of this section, "qualifying
14 opioid therapy patient" means:

15 1. A patient requiring opioid treatment for more than three (3)
16 months;

17 2. A patient who is prescribed benzodiazepines and opioids
18 together for more than one twenty-four-hour period; or

19 3. A patient who is prescribed a dose of opioids that exceeds
20 one hundred (100) morphine equivalent doses.

21 ~~K.~~ L. Nothing in ~~the Anti-Drug Diversion Act~~ this section shall
22 be construed to require a practitioner to limit or forcibly taper a
23 patient on opioid therapy. The standard of care requires effective
24 and individualized treatment for each patient as deemed appropriate

1 by the prescribing practitioner without an administrative or
2 codified limit on dose or quantity that is more restrictive than
3 approved by the Food and Drug Administration (FDA).

4 SECTION 2. This act shall become effective November 1, 2026.

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6 COMMITTEE REPORT BY: COMMITTEE ON HEALTH AND HUMAN SERVICES
7 OVERSIGHT, dated 04/16/2026 - DO PASS.
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